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CONFERENCE ABSTRACT

Cultivating a common language for change: Co-designing mental health conservations with community-based providers across Canada

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Introduction/Background: Systemic ageism, overmedicalization, stigma, and misunderstandings about mental health as a unidimensional or dichotomous construct, contribute to aging Canadians holistic needs going unnoticed, undiscussed, and ultimately unmet. A common language is needed by home and community care providers to leverage their routine care interactions for meaningful conversations about mental health. This national study aimed to co-design and test an evidence-based approach to mental health conversations with home and community care providers in urban and rural communities across Canada.

Approach: Guided by a pan-Canadian working group of experts-by-experience (n=25), this multi-phase research study builds on previous community-based priority-setting work. The project emphasizes intersectionality to understand meaningful differences in mental health experiences to create an inclusive and evidence-based approach to dialogue. Using a participatory, mixed methods design, we have continuously engaged experts-by-experience in all phases of the project, including older adults, caregivers, and health and social care providers from all 3 provinces and territories in Canada, with all data collection and deliverables available in both English and French. Phase included online workshops and surveys with older adults, caregivers, and health and social care providers across Canada (n=27) to adapt an existing visual model of mental health for use with older adults. In phase 2, health and social care providers (n=84) participated in seven co-design workshops in rural and urban communities across three Canadian provinces. Care providers engaged in interactive gamestorming activities by ideating, prototyping and role-playing various approaches to facilitate mental health conversations at the point of care. Workshop artefacts and transcripts were analyzed using framework analysis. Phase 3 involves piloting the implementation of the Mental Health Conversations Initiative in collaboration with 5 home and community care organizations to understand feasibility and preliminary outcomes for client, caregiver and provider participants.

Results: Phases and 2 led to the development of the Mental Health Conversations Initiative, a dynamic, evidence-based approach to mental health dialogue in community-based settings. A core element of the initiative and output from Phase is the Mental Health Continuum for Aging

Canadians (MHCAC), which supports use of common language during mental health conversations and illustrates the multi-dimensional nature of mental health. Leveraging the MHCAC, the co-design process from Phase 2 resulted in three outputs designed to be used flexibly to respond to individual needs and preferences :) a Conversation Guide to support care provider decision -making during mental health conversations; 2) a Mental Health Conversations Toolkit with resources based on the MHCAC; and 3) an Implementation Framework for home and community care organizations, including guidance for provider training and education. At the time of the conference, we expect Phase 3 implementation will be underway.

Implications: Discipline-agnostic, non-clinical mental health conversations are necessary first steps towards community-enabled population mental health and wellbeing. The Mental Health Conversations Initiative will support adoption of a common language and understanding of mental health among care providers, older adults, and caregivers, which may begin to destigmatize mental health and promote individual help-seeking behaviours and linkages to available supports, care and treatments. Leveraging findings from Phase 3, next steps will involve exploring further spread and scale of the Mental Health Conversations Initiative in home and community care settings across Canada. Knowledge mobilization efforts have also indicated potential applicability of the Mental Health Conversations Initiative beyond home and community care, including facility-based long-term care settings, training and education programs for clinicians, and broader public awareness initiatives.